

Barcarse Psychological Services

Psychological Clinic Services
Psychological Assessment Section
Sampaloc, Manila, Philippines

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PSYCHOLOGICAL ASSESSMENT REPORT "Confidential"

IDENTIFYING INFORMATION:

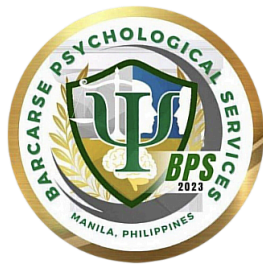
Name:
Age:
Date of Birth:
Sex:
Residential Address:
Date of Report:

ASSESSMENT TOOLS/PROCEDURE	DATE OF ADMINISTRATION
MacKenzie Danvey Mental Ability Test (MD5)	May 09, 2026
Purdue Non-Language Test (PNLT)	May 09, 2026
BarOn EQ Inventory: Short Version	May 11, 2026
Masaklaw na Panukat ng Loob (MAPA ng Loob)	May 11, 2026
Basic Personality Inventory (BPI)	May 11, 2026
The Personality Inventory for DSM-5 (PID-5)—Adult	May 11, 2026
Aggression Questionnaire (AQ)	May 11, 2026
DSM-5-TR Self-Rated Level 1 Cross-Cutting Symptom Measure	May 09, 2026
Patient Health Questionnaire (PHQ-9)	May 09, 2026
Generalized Anxiety Disorder Screening (GAD-7)	May 09, 2026
Depression, Anxiety, and Stress Scale (DASS-21)	May 09, 2026
Draw-A-Person Test (DAPT)	May 11, 2026
Sach's Sentence Completion Test	May 11, 2026

GENERAL OBSERVATION:

The client was appropriately groomed and casually dressed during the assessment. He wore eyeglasses and had a noticeable black eye on his right eye. He was oriented to person, place, and time, and was cooperative throughout the evaluation. Speech was generally coherent, relevant, and goal-directed, with normal rate and volume. Mood appeared subdued, with affect observed to be restricted or low in intensity, although occasional appropriate laughter and humor were present during the interaction.

Thought processes were generally logical and organized. No overt signs of perceptual disturbances, delusions, or disorganized thinking were observed during the assessment. The client demonstrated adequate comprehension of questions and was able to provide detailed responses regarding his personal history and experiences. At times, particularly when discussing emotionally significant events, he would pause and stare blankly momentarily before resuming the conversation.



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Insight and judgment appeared fair, as evidenced by his awareness and concern regarding confidentiality and his appropriate engagement during the assessment process.

CASE BACKGROUND:

Reason for Referral

The client sought psychological consultation following a recent breakup that resulted in severe emotional distress, anxiety, panic attacks, and recurring suicidal thoughts. He was previously advised by a counselor from the Far Eastern University - Institute of Technology to undergo psychological assessment and consultation to better understand his emotional difficulties, behavioral patterns, and relationship concerns. The client expressed a desire to understand why he repeatedly engages in destructive behaviors despite wanting stable relationships and personal improvement.

Signs and Symptoms Presented

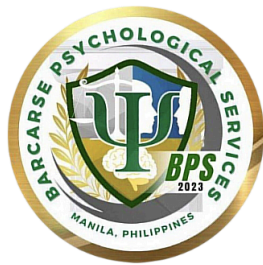
The client reported experiencing anxiety, panic attacks, irregular sleep patterns, fluctuating appetite, emotional instability, and persistent overthinking. He shared that he frequently experiences intense emotional reactions during relationship conflicts, which often lead to self-harming behaviors and suicidal thoughts. He described a pattern of impulsive actions, guilt, jealousy, emotional dependence, and fear of abandonment. The client also acknowledged difficulties with trust, emotional regulation, and coping with rejection. He described himself negatively, referring to himself as manipulative, dishonest, and emotionally destructive, although he also expressed remorse and self-awareness regarding his behaviors.

Family History

The client grew up in a separated family setup since birth. According to him, his parents agreed to separately raise one child each despite already being separated. He was primarily raised by his paternal grandmother, while his older sister was raised on his mother's side. He shared that he was frequently compared to his older sister, who was often praised, which contributed to feelings of inadequacy and emotional neglect. His relationship with his mother has remained strained, although they are currently attempting to reconnect. In contrast, he maintains regular communication with his father, who continues to support him in some aspects, including housing. He also shared that his maternal grandmother was diagnosed with schizophrenia, and he has a cousin diagnosed with an anxiety-related disorder.

Educational Background

The client experienced multiple academic disruptions throughout his schooling. During high school, he began cutting classes and engaging in substance use, which negatively affected his academic performance. He attended different colleges but struggled to maintain consistency due to behavioral concerns, emotional distress, and relationship conflicts. At present, he is a freshman irregular Information



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Technology student at FEU while simultaneously working in a BPO company to partially financially support his studies. Despite repeated setbacks, the client continues to express a desire to finish his education and improve his life circumstances.

Relationship History

The client has a long history of unstable and emotionally intense romantic relationships. He described patterns of repeated breakups, reconciliation attempts, jealousy, mistrust, infidelity, emotional dependence, and self-destructive behaviors during conflicts. Several of his relationships involved mutual cheating, emotional manipulation, threats of self-harm, and difficulty setting boundaries. He also shared engaging in multiple casual sexual relationships, flings, and one-night stands during periods of emotional instability and substance use. Despite expressing a strong desire to settle down and maintain stable relationships, he often found himself repeating maladaptive behaviors that later resulted in guilt, regret, and emotional distress. His most recent relationship particularly intensified his emotional struggles, leading to severe suicidal thoughts, increased anxiety, and significant self-blame following their breakup.

Medical History

The client reported having an irregular sleep cycle and usually sleeps for only a few hours daily, although he believes it is enough for him to function. He also experiences fluctuating appetite, recently eating as little as one meal per day or sometimes none at all. He was admitted to a rehabilitation facility in 2017 for approximately six to seven months due to self-harming behaviors and substance use concerns. During this time, he was diagnosed with dysthymia and was prescribed psychiatric medications, including escitalopram and syclop. However, the client currently expressed hesitation toward psychopharmacological treatment if alternative interventions are possible.

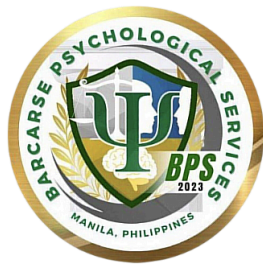
Trauma History

The client described his childhood as traumatic and emotionally unstable. He experienced feelings of rejection, emotional neglect, family instability, and constant comparison while growing up. Throughout adolescence and adulthood, he encountered repeated interpersonal conflicts, betrayal, unstable relationships, self-harming episodes, suicidal behaviors, and substance use. He also described emotional crises related to abandonment, rejection, and fear of being left alone. His experiences appear to have significantly affected his emotional regulation, self-worth, coping abilities, and interpersonal functioning.

ASSESSMENT RESULTS AND INTERPRETATION:

INTELLECTUAL FUNCTIONING

The results from the assessment showed that the client has overall average intellectual functioning. He possesses adequate capacity for learning, problem-solving, comprehension, and adapting to new information. He is capable of understanding complex situations and reflecting on his own



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thoughts, emotions, and behaviors. His ability to maintain employment while pursuing college studies also suggests practical thinking skills and functional intellectual abilities despite ongoing emotional difficulties.

In addition, he appears to have difficulty with emotional control, impulse regulation, and consistent decision-making, particularly during periods of emotional distress. Although he is capable of insight and self-awareness, his intense emotional reactions often interfere with his judgment and ability to think through consequences before acting. This is reflected in his history of difficulty maintaining consistency in school and personal responsibilities. He may also struggle with planning, sustaining long-term goals, and managing stress effectively, especially when overwhelmed by feelings of rejection, abandonment, or conflict.

Despite these difficulties, the client still possesses strengths in the form of self-reflection and motivation for self-improvement. He is able to recognize problematic patterns in his behavior and expresses a genuine desire to change and achieve stability in his life. His continued efforts to pursue education and work toward personal growth suggest preserved cognitive potential and the capacity to improve functioning when provided with appropriate emotional support, structure, and intervention.

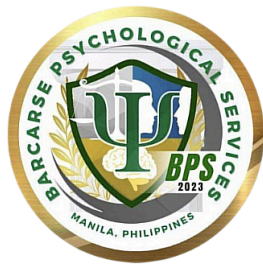
EMOTIONAL FUNCTIONING

The client exhibits severe emotional distress characterized by anxiety, depression, emotional instability, impulsivity, and poor emotional regulation. He experiences intense emotional reactions, particularly during relationship conflicts or situations involving rejection and abandonment. These emotional experiences are often accompanied by panic attacks, overthinking, self-blame, guilt, suicidal thoughts, and self-harming behaviors. He also reports irregular sleep, fluctuating appetite, emotional exhaustion, and persistent feelings of sadness and emptiness.

The client appears highly emotionally sensitive and reactive, with difficulty managing strong emotions once triggered. He may rapidly shift between feelings of anger, sadness, fear, and guilt, which contributes to impulsive decisions and unstable behaviors during times of distress. There are also signs of low self-esteem, shame, emotional dependency, and internal conflict regarding his behaviors and relationships. Although he describes himself negatively and expresses regret about past actions, he also demonstrates self-awareness and insight into his emotional struggles.

SOCIAL FUNCTIONING

The client experiences significant difficulties in social and interpersonal functioning, particularly in forming and maintaining stable, healthy relationships. Although he strongly desires affection, connection, and emotional security, his fear of abandonment, rejection, and being left alone heavily influences how he relates to others. He tends to become emotionally dependent in relationships and may



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react intensely to perceived rejection, criticism, or emotional distance. These patterns often contribute to his feelings of jealousy, mistrust, conflict, and having unstable interpersonal interactions.

The client also appears to struggle with trust and vulnerability. While he values dependable and caring relationships, he often feels emotionally unsupported and misunderstood by others. He may have difficulty openly expressing emotional needs and instead rely on indirect expressions of frustration such as withdrawal, avoidance, silence, or passive-aggressive behaviors. At times, he may become verbally reactive or argumentative when emotionally overwhelmed, especially during interpersonal conflict.

His family experiences, particularly the separation of his parents, emotional neglect, and repeated comparison with his older sister, appear to have contributed to long-standing feelings of inadequacy and insecurity in relationships. These unresolved emotional experiences continue to affect his self-worth and attachment patterns. Despite these difficulties, the client still shows the ability to maintain some meaningful connections, continues to seek reconciliation with family members, and expresses a genuine desire to build stable and loving relationships in the future.

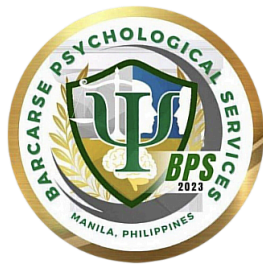
DEFENSE MECHANISM

The client primarily uses avoidance, emotional withdrawal, fantasy, emotional suppression, and self-harming behaviors as ways of coping with distress. When faced with painful emotions or interpersonal conflict, he may distance himself emotionally, avoid directly confronting problems, or escape into thoughts, fantasies, or distractions to temporarily relieve emotional discomfort. He also appears to rely on rationalization and overthinking in an attempt to make sense of his behaviors and emotional experiences. Although he demonstrates self-awareness and insight into his actions, he may struggle to translate this insight into consistent behavioral change, especially during emotionally intense situations.

There are also indications of emotional overcontrol combined with impulsive emotional release. The client may attempt to suppress feelings of anger, sadness, guilt, or insecurity for long periods, but once emotionally overwhelmed, he may react impulsively through verbal aggression, self-destructive behaviors, self-harm, substance use, or emotionally reactive actions within relationships. Passive-aggressive tendencies, mistrust, and defensiveness may also emerge when he feels criticized, abandoned, or emotionally unsafe. While these defense mechanisms may help him temporarily manage distress and maintain emotional control, they also contribute to unresolved emotional conflict, unstable relationships, and difficulty developing healthy emotional expression.

RISK

The client currently presents with significant psychological risk factors due to severe depression, anxiety, emotional instability, suicidal thoughts, history of self-harm, and substance use concerns. His emotional distress appears strongly connected to fears of abandonment, unstable relationships,



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unresolved childhood trauma, and chronic feelings of inadequacy and rejection. During periods of emotional conflict or perceived loss, he may become highly impulsive and vulnerable to engaging in self-destructive behaviors, including self-harm, suicidal ideation, emotional outbursts, and maladaptive coping behaviors.

His history of rehabilitation, previous psychiatric treatment, unstable interpersonal relationships, and difficulty regulating emotions further increase his vulnerability during stressful situations. The client also demonstrates severe emotional dependence and difficulty coping with rejection, which may place him at higher risk for emotional crises following relationship conflicts or breakups. Sleep disturbance, poor appetite, emotional exhaustion, and severe overthinking may further worsen his psychological functioning if left untreated.

CLINICAL IMPRESSIONS:

The client presents symptoms consistent with his previous diagnosis of **Persistent Depressive Disorder**, with current **Major Depressive Episode**, consistent with *“double depression”* diagnosis. As such, at present, the client feels low self-worth, guilt, hopelessness, emotional exhaustion, persistent emotional distress, loss of pleasure, suicidal ideation on top of his chronic low mood.

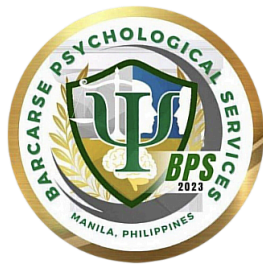
At the same time, the client’s current presentation is also consistent with **Borderline Personality traits**, as evidenced by significant emotional instability, intense fear of abandonment, unstable and conflict-ridden interpersonal relationships, impulsivity, and self-harming behaviors. The client demonstrates a long-standing pattern of interpersonal dysfunction, emotional reactivity, and maladaptive coping behaviors, particularly during situations involving rejection, conflict, or perceived abandonment. These difficulties appear to be deeply rooted in traumatic childhood experiences, family instability, emotional neglect, and repeated interpersonal stressors.

Additionally, further assessment is also warranted to rule out a possible substance use disorder, given the client’s history of substance use. Further evaluation is also warranted to rule out the presence of other psychotic disorders or psychotic-related symptoms, particularly considering the client’s previous prescription of Syclop and family history of severe mental illness.

RECOMMENDATIONS:

Based on the findings, the client is recommended to:

- Engage in regular psychotherapy sessions with a licensed psychologist, particularly using approaches focused on emotion regulation, interpersonal functioning, and distress tolerance.
- Establish a safety plan addressing suicidal thoughts, self-harming behaviors, emotional triggers, coping strategies, and emergency support contacts.



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- *Conduct further assessment* to determine the extent of persistent depressive symptoms, including appropriate specifiers and severity, and to rule out substance use disorder and monitor possible relapse risks given the client's history. It will also help determine possible psychotic-related symptoms or other psychotic disorders, especially considering the previous prescription of Syclop and family history of severe mental illness.
- Maintain consistent sessions with his school counselor to provide ongoing emotional support, academic guidance, monitoring, and assistance in managing school-related stressors and adjustment difficulties.
- Improve emotion regulation and impulse control skills, particularly during situations involving rejection, abandonment, jealousy, or interpersonal conflict.
- Develop healthier relationship boundaries and communication patterns to reduce emotional dependence, manipulation, and unstable interpersonal dynamics.
- Encourage consistent sleep, eating, and self-care routines to improve emotional and physical stability.
- Reduce reliance on maladaptive coping behaviors such as emotional withdrawal, avoidance, substance use, impulsive behaviors, and self-destructive reactions.
- Strengthen support systems by encouraging healthy communication and reconnection with trusted family members or supportive individuals.
- Continue pursuing educational and occupational goals, while balancing responsibilities with emotional recovery and mental health needs.

PREPARED BY:

Name of Psychometrician

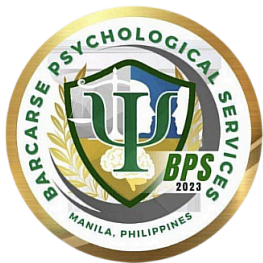
Barcarse Psychological Services - Psychological Assessment Section
Psychometrician License No.:
Valid Until:

APPROVED BY:

Name of Psychologist

Clinical Psychologist

Barcarse Psychological Services - Psychological Assessment Section
Psychologist License No.
PTR No.



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MENTAL HEALTH CERTIFICATE

This is to certify that _____ residing in Quezon City, Metro Manila has undergone a comprehensive psychological assessment with the undersigned on May 9 & 11, 2026 at Barcarse Psychological Services Clinic on the 2nd Floor, Unit 307, Dioresa Plaza Building, 1521 Dapitan Street, corner Lacson Avenue, Sampaloc, Manila. Based on the assessment, the client meets the criteria for

Double Depression- Persistent Depressive Disorder with Major Depressive Episode
with Borderline Personality traits

This mental health certificate is being issued this 19th day of May 2026, as a requirement for the issuance of a Person with Disability ID and is valid for 6 months only. Should you require further information, please do not hesitate to contact our clinic.

Noted by:

Name of Psychologist

Clinical Psychologist

Barcarse Psychological Services - Psychological Assessment Section

Psychologist License No.

PTR No.